SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Carboplatin + fluorouracil after nasopharyngeal chemoradiotherapy

Adjuvant chemotherapy after chemoradiotherapy for locally advanced stage III or IV nasopharyngeal cancer when cisplatin cannot be used or tolerated. · NCCP 00552 · version 6 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This chemotherapy is an additional treatment after chemoradiotherapy for nasopharyngeal cancer. It aims to treat cancer cells that may remain. The team explains the expected benefit and burden for you, including recovery from the earlier treatment.

Schedule
After recovery, adjuvant chemotherapy is planned to begin 4 weeks after completion of weekly carboplatin with radiotherapy (NCCP 00419). Carboplatin is given on Day 1 and fluorouracil over Days 1–4, every 28 days for up to 3 cycles. The treating team confirms readiness and dates.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Carboplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Low platelets and bleeding
- ☐ Anaemia and fatigue
- ☐ Nausea and vomiting
- ☐ Infusion or allergic reactions
- ☐ Kidney, nerve or hearing effects

Fluorouracil
ANTIMETABOLITE CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Mouth and throat soreness
- ☐ Diarrhoea and dehydration
- ☐ Painful palms or soles
- ☐ Heart problems including chest pain
- ☐ Rare severe neurological or early excessive toxicity

Discuss the individual aim, expected benefit, alternatives and uncertainties; common, serious and lasting effects; infusion reactions and infection; fertility and pregnancy; treatment burden; support medicines and urgent contacts. NCCP identifies carboplatin in this nasopharyngeal indication as an off-label use. Discuss why it is being offered, the evidence, alternatives and uncertainties, and document consent under local policy.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00552 v6.

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